SPECIALIZED MEDICAL

Mobile Cardiac Telemetry Landing Page SEO / AEO Developer Task List

Step-by-step corrections, acceptance tests, and production launch requirements



STAGING PAGE	https://zesty-pothos-c8949b.netlify.app/mobile-cardiac-telemetry-mct
PRODUCTION TARGET	https://specialized-med.com/mobile-cardiac-telemetry-mct/
PREPARED FOR	Steven Burns, CEO - Specialized Medical LLC
DATE	October 13, 2026

CURRENT STATUS: HOLD FOR CORRECTIONS

Do not move this page to the live Specialized Medical website until Stages 0 through 6 have passed their acceptance gates. The current page is a strong SEO/AEO foundation, but it contains brand, trust, form, canonical, staging-indexing, and clinical-review issues that must be corrected.

How to Use This Task List

Complete the stages in order. Each task has a responsible role, priority, exact action, and pass test. A stage is complete only when every blocker and high-priority item in that stage passes. Save evidence for each acceptance test, including live URLs, validation results, screenshots, and test-submission confirmations.

ROLE	RESPONSIBILITY
Web Developer	HTML, CSS, JavaScript, image delivery, forms, redirects, headers, accessibility and deployment.
SEO/AEO Developer	Metadata, canonicalization, crawl/index controls, schema, internal linking, sitemap, crawler access and search validation.
Content/Clinical Review	Claim accuracy, evidence support, CPT wording, safety statements, author and reviewer attribution.
Analytics/QA	Conversion events, form testing, browser/mobile QA, accessibility, performance and post-launch monitoring.

Priority definitions

[ ] **BLOCKER:** The page must not launch until fixed.

[ ] **HIGH:** Required for the first production release.

[ ] **MEDIUM:** Complete before launch when feasible; otherwise schedule immediately after launch.

Final launch standard

[ ] No visible competitor equipment, competitor branding, or implied affiliation.

[ ] No internal editorial instructions, placeholder reviewer text, unsupported outcomes, or reimbursement promises.

[ ] No staging-domain URLs in canonical tags, metadata, schema, sitemaps, or social-sharing markup on production.

[ ] All important answers exist in visible HTML, not only in graphics or structured data.

[ ] The page works on mobile, submits leads correctly, and is indexable only on the production domain.

Stage 0 - Contain the Staging Page

Purpose: prevent the temporary Netlify copy from competing with the final Specialized Medical page or being surfaced to search users.

	Password-protect the staging site	Web Dev	BLOCKER

Action
Use Netlify access control, basic authentication, or an equivalent staging gate. Do not rely on robots.txt alone to keep the page private.

Pass test
An unauthenticated browser receives an authentication challenge or cannot retrieve the page content. The access control applies to all staged landing pages and assets where practical.

	Add staging noindex controls	SEO/AEO	BLOCKER

Action
Until password protection is confirmed, serve both an X-Robots-Tag response header and a page-level robots directive containing **noindex, nofollow**. Remove the current **index, follow** instruction.

Pass test
View-source shows noindex and the HTTP response shows the matching X-Robots-Tag. URL Inspection or an equivalent crawler test confirms the directive is discoverable.

	Remove staging from discovery feeds	SEO/AEO	HIGH

Action
Do not include Netlify URLs in the production XML sitemap, internal production links, Organization schema, social profiles, or marketing materials. If a staging sitemap exists, keep it private.

Pass test
Search the production codebase and generated sitemap for **zesty-pothos-c8949b.netlify.app**; the result count is zero.

	Preserve a rollback copy	Web Dev	HIGH

Action
Create a versioned backup of the current page, CSS, JavaScript, images, form configuration, and schema before applying changes.

Pass test
A dated, restorable version exists and a developer confirms the page can be redeployed from it.

STAGE GATE	The staging page is access-controlled, carries noindex while accessible, and is absent from all production discovery paths.

Stage 1 - Correct Brand, Imagery and Trust Signals

	Replace the Philips hero image	Web Dev	BLOCKER

Action
Replace **images/landing/mobile-cardiac-telemetry-mct.jpg** with the approved Specialized Medical MCT hero. Use the supplied 1430 x 550 WebP as the primary source and JPG as fallback. Keep the equipment on the right and copy space on the left.

Pass test
No Philips name, device, logo, or other competitor mark appears in the page, image metadata, CSS, source filenames, alt text, Open Graph image, or social preview.

Approved implementation files

```
Primary: mobile-cardiac-telemetry-mct-hero.webp (1430 x 550, approximately 22 KB)
Fallback: mobile-cardiac-telemetry-mct-hero.jpg (1430 x 550)
Alt text: Specialized Medical S-Patch mobile cardiac telemetry system with a connected phone displaying live ECG data.
```

Remove or rebuild the overview graphic

Content + Web | BLOCKER

Action
The overview graphic currently contains high-risk language such as **better outcomes**, **immediate alerts**, **never miss**, **proven results**, and similar absolute claims. Replace those statements with verifiable service features, or remove the graphic until corrected.

Pass test
Every visible claim in the graphic is supportable and matches visible page copy. No outcome guarantee, prevention claim, instantaneous-alert promise, or unverified market-leadership statement remains.

Remove internal editorial instructions

Content | BLOCKER

Action
Delete the visible labels **Compliance wording** and **Required safety statement**. Retain necessary safety information as polished visitor-facing copy without exposing drafting instructions.

Pass test
A search of rendered text and source returns zero matches for those two labels. Safety information remains visible in normal page language.

Fix reviewer attribution

Clinical | BLOCKER

Action
Remove **Pending authorized physician review** and the promise that a reviewer will be added later. Either obtain approval from a named qualified reviewer and publish accurate credentials/date, or use the approved editorial-review wording in Appendix A without claiming medical review.

Pass test
The reviewer statement is factually true, contains no placeholder language, and the displayed updated date matches the actual content review.

Remove unsupported absolute claims

Content | HIGH

Action
Review the entire page for **continuous**, **uninterrupted**, **immediate**, **prevents**, **protects**, **guarantees**, **never miss**, and outcome language. Use qualified wording that accounts for battery, Bluetooth, cellular connectivity, and clinician workflows.

Pass test
A final claim audit is signed off by Specialized Medical. Connectivity and notification language is consistent throughout the page and graphics.

STAGE GATE	All imagery and wording accurately represent Specialized Medical's system. No competitor brand, internal note, placeholder review statement, or unsupported outcome claim remains.

Stage 2 - Repair Conversion Flow and Lead Form

	Fix every CTA anchor	Web Dev	BLOCKER

Action
The page CTAs must scroll to the form on the same MCT page. Correct any base-URL or JavaScript behavior that resolves **#cta-form** to the staging or production homepage. Use a same-page fragment that remains on the MCT URL.

Pass test
Click every hero, mid-page, and bottom CTA on desktop and mobile. The URL remains the MCT page and the form receives keyboard focus or is positioned visibly below the fixed header.

	Establish one primary CTA	Content + UX	HIGH

Action
Use **Start a No-Risk MCT Pilot** as the primary solid button. Use **Request an MCT Demonstration** as the secondary button. Keep this hierarchy consistent throughout the page.

Pass test
All CTA labels, styles, analytics event names, and form-interest options use the same terminology.

	Remove the premature thank-you message	Web Dev	BLOCKER

Action
Replace the form's opening sentence beginning **Thank you** with a pre-submission invitation. Display the thank-you state only after a confirmed successful submission.

Pass test
On initial load there is no thank-you message. On successful submission a confirmation appears. On failure an accessible error message appears and entered values are preserved where safe.

	Harden and clarify the form	Web Dev	BLOCKER

Action
Confirm the Web3Forms destination, domain restrictions, spam controls, success behavior, and owner notification. Add a visible no-PHI warning and Privacy Policy link. Do not send form-field values to analytics. Configure a defined success page or in-page success state instead of a blank redirect.

Pass test
Test submissions from desktop and mobile arrive at the correct Specialized Medical inbox. Spam protection works. No patient data request exists. The browser network log contains no form PII in analytics calls.

	Improve lead qualification without adding friction	UX	HIGH

Action
Use: Name, Practice/Organization, Role, Business Email, Phone, State, Interest (Pilot / Demonstration / Both), Preferred Contact Method, and optional approximate monthly monitoring volume. Clearly mark optional fields.

Pass test
Labels remain visible, required fields are announced, autocomplete attributes are correct, tab order is logical, and the form can be completed at 320 CSS pixels wide.

	Track conversion actions	Analytics	HIGH

Action
Create GA4 or equivalent events for primary CTA click, demonstration click, form start, successful form submission, phone click, and physician-portal click. Do not include names, emails, phone numbers, organizations, or other user-entered values.

Pass test
Events fire once per intended action in debug mode, use documented names, and the form-submit event fires only after confirmed success.

STAGE GATE	Every CTA reaches the correct form, the form submits successfully, the confirmation logic works, no PHI is requested, and conversion events are verified without PII.

Stage 3 - Production URL and On-Page SEO

	Use one clean production URL	SEO/AEO	BLOCKER

Action
Publish at **https://specialized-med.com/mobile-cardiac-telemetry-mct/**. Choose one trailing-slash convention for the site. Redirect all alternate versions, including any **.html** path, queryless duplicate, HTTP version, and host variation, to the chosen URL.

Pass test
Each alternate URL returns one 301 hop to the production URL. The production page returns 200. No redirect chain or duplicate 200 response exists.

	Install a self-referencing canonical	SEO/AEO	BLOCKER

Action
On production, set the canonical to the exact preferred Specialized Medical URL. Replace all Netlify canonical values and all schema/social URLs that contain **.html** or the staging host.

Pass test
View-source contains one canonical. It matches the final browser URL exactly. A source search returns zero staging-domain references.

	Set final title and description	SEO/AEO	HIGH

Action
Use a unique title and conversion-focused description. Recommended copy appears in Appendix A. Avoid title stuffing and reserve all-caps LIVE for limited branded use in visible copy.

Pass test
There is one non-empty title and one meta description. They are unique to this page and render cleanly in desktop and mobile snippet-preview tools.

	Maintain one clear H1 and logical headings	Content + Web	HIGH

Action
Keep one H1 focused on Mobile Cardiac Telemetry. Use H2 sections for definition, clinical uses, comparison, technology, workflow, CPT considerations, FAQ, and next step. Do not skip heading levels for styling.

Pass test
The DOM contains one H1. A heading-outline test shows a logical hierarchy and each heading describes the answer that follows.

	Add answer-first introductory copy	Content	HIGH

Action
Directly under the H1, explain what the service is, who it serves, monitoring duration, live transmission, attended surveillance, prescribed notifications, patient support, and physician-ready reporting in concise visible text.

Pass test
A reader can understand the offer and next step without scrolling. The core service facts are HTML text, not embedded in the image.

	Build the internal-link cluster	SEO/AEO	HIGH

Action
Link to this page from the homepage, Services hub, S-Patch page, Live ECG page, Post-TAVR page, FAQ/resources, and relevant clinical stories. Link from this page to Holter, Long-Term Holter, Event Monitoring, S-Patch, Post-TAVR, Services, and Contact/Pilot pages using descriptive anchors.

Pass test
A crawl report shows no orphan status. All internal links return 200 or the intended single-hop redirect. Anchors describe destinations and are not repeated unnaturally.

STAGE GATE	The production URL is unique, canonicalized, correctly titled, logically structured, internally linked, and free of staging-domain references.

Stage 4 - AEO Content and Structured Data

	Create a connected JSON-LD graph	SEO/AEO	HIGH

Action
Use valid JSON-LD for **MedicalWebPage**, **Service**, **BreadcrumbList**, and **FAQPage**. Connect entities using stable production **@id** values. Reference Specialized Medical as provider/publisher and use production URLs only.

Pass test
Schema.org Validator reports no errors. Google's Rich Results Test parses eligible types. All URLs resolve and all marked-up content is visible on the page.

	Match FAQ schema to visible FAQ content	SEO/AEO	BLOCKER

Action
Every FAQ question and accepted answer in JSON-LD must match a visible, accessible question and answer on the page. Do not hide extra schema-only FAQs. Keep approximately 8 to 12 high-value questions in the primary accordion.

Pass test
A line-by-line comparison confirms visible and structured FAQ content matches. Accordion content is present in the DOM and accessible without requiring a search crawler to click.

	Add concise answer blocks	Content	HIGH

Action
Under important question-style headings, begin with a direct 40 to 70 word answer before adding detail. Prioritize: What is MCT? How does it differ from Holter? How long does it monitor? How do alerts work? What does the practice receive? How does a pilot begin?

Pass test
Each priority question has a self-contained, factually qualified answer that can be quoted without losing essential context or safety limitations.

	Strengthen entity and authorship signals	Content + SEO	HIGH

Action
Show the page author, role, organization, reviewed/updated date, editorial policy, phone, address, and contact path. Use **reviewedBy** only if a named reviewer actually approved the page. Link Organization identity consistently across the site.

Pass test
Visible attribution and JSON-LD agree. Dates are real. The organization name, address, phone, logo URL, and canonical domain are consistent.

	Allow AI search crawling on production	SEO/AEO	HIGH

Action
Confirm production robots.txt does not block OAI-SearchBot from the MCT page or required assets. If explicit rules are used, allow OAI-SearchBot. Treat GPTBot as a separate training-policy choice; allowing GPTBot is not required for ChatGPT search visibility.

Pass test
A robots tester confirms OAI-SearchBot can fetch the production URL, hero image, CSS, and rendered content. The staging domain remains protected.

Recommended production robots.txt addition

```
User-agent: OAI-SearchBot
Allow: /
```

	Do not promise FAQ rich results	SEO/AEO	MEDIUM

Action
Keep FAQPage markup for accurate machine-readable structure, but do not represent it as a guaranteed Google rich-result feature. Google limits FAQ rich-result visibility primarily to well-known authoritative health and government sites.

Pass test
Developer notes and reporting distinguish valid schema from guaranteed visual search treatment. No contract, report, or page claim promises rich-result display.

STAGE GATE	Structured data validates, visible answers match marked-up answers, entity/reviewer facts are consistent, and OAI-SearchBot can access the production page.

Stage 5 - Performance, Mobile and Accessibility

	Deliver the hero efficiently	Web Dev	HIGH

Action
Use the 22 KB WebP as the primary hero image with explicit width and height, correct object positioning, preload or fetchpriority=high, and a JPG fallback. Do not lazy-load the above-the-fold hero.

Pass test
The hero does not cause layout shift, remains sharp on common screens, and is the intended Largest Contentful Paint element. No oversized Philips asset is requested.

Optimize below-the-fold media	Web Dev	HIGH

Action
Compress and resize all images to their rendered dimensions, provide modern formats, add width/height, and lazy-load non-critical media. Remove unused image variants and duplicate asset requests.

Pass test
A network audit shows no image materially larger than its display need, no missing dimensions, and no below-the-fold image loaded eagerly without reason.

Meet Core Web Vitals targets	Web Dev	HIGH

Action
Target LCP <= 2.5 seconds, INP <= 200 milliseconds, and CLS <= 0.1 at the 75th percentile. Reduce unused CSS/JavaScript, defer non-critical scripts, and keep form/accordion interactions responsive.

Pass test
PageSpeed Insights and Search Console field data meet the good thresholds when sufficient data exists. Until then, mobile lab tests show no major performance blocker and Lighthouse performance is documented.

Complete responsive QA	QA	BLOCKER

Action
Test 320, 375, 390, 768, 1024, 1440 and 1920 CSS-pixel widths. Confirm hero crop, headings, comparison tables, accordions, CTAs, form, footer, sticky navigation, and external citations remain usable.

Pass test
There is no horizontal page scroll, clipped content, covered anchor target, overlapping button, unreadable table, or off-screen form control.

Meet accessibility basics	Web Dev + QA	HIGH

Action
Ensure keyboard access, visible focus, semantic headings, programmatic labels, error identification, accordion states, contrast, descriptive links, alt text, and a skip-to-content mechanism. Decorative images must use empty alt text.

Pass test
Automated accessibility testing returns no serious or critical violations. A manual keyboard pass completes all navigation, FAQs, links, and form submission without a mouse.

STAGE GATE	The page is fast, stable, responsive, keyboard-accessible, and free of critical accessibility or mobile-layout defects.

Stage 6 - Medical, Evidence and Billing Review

Verify every cited claim	Clinical + Content	BLOCKER

Action
Create a claim-to-source worksheet for monitoring duration, attended surveillance, treatment-response use, diagnostic yield, post-TAVR discussion, connectivity, battery, water resistance, notification workflow, and reporting. Do not extend study findings to Specialized Medical outcomes unless directly supported.

Pass test
Every material clinical or technical claim has a source or internal product verification. Study-specific results are identified as study-specific and not presented as guaranteed outcomes.

	Qualify live transmission and alert language	Clinical + Content	BLOCKER

Action
State that ECG data is transmitted while the study is active and connectivity is available. Use **physician-prescribed notification** rather than **immediate alert** unless a documented service-level definition supports timing.

Pass test
All page text, graphics, metadata, schema and FAQs use consistent qualified wording. No part of the page implies guaranteed uninterrupted transmission.

	Correct CMS and payer framing	Billing + Content	BLOCKER

Action
Do not present a Local Coverage Determination or billing article as a universal national rule. Identify that coverage, medical necessity, prior authorization, frequency and documentation requirements vary by payer, plan and Medicare Administrative Contractor jurisdiction.

Pass test
The CPT section includes the approved qualified language in Appendix A and no sentence guarantees coverage, payment, profitability, or eligibility.

	Review CPT 93228 and 93229 wording	Billing + Content	BLOCKER

Action
Define the professional and technical components without stating that both codes are always needed or always billable by the practice. Explain that the reporting entity depends on services performed, contractual arrangement, payer rules, and documentation.

Pass test
A qualified billing reviewer approves the final wording. The page does not instruct a practice to bill a component it did not perform or is not permitted to report.

	Keep emergency and medication limitations visible	Clinical	HIGH

Action
Retain clear language that MCT is not emergency response, does not replace 911, and does not replace medication-specific monitoring, laboratory tests, 12-lead ECG, QT assessment, inpatient observation, labeling, guidelines, or clinician judgment.

Pass test
The safety statements are plainly written, visible near relevant content and the form, and do not appear under internal-instruction headings.

	Refresh authorship and dates	Content	HIGH

Action
Update the displayed content date when the revised page is approved. Retain Steven M. Burns as business/service author if accurate. Add clinical reviewer credentials only after completed review and documented authorization.

Pass test
Visible byline, reviewer, dates and JSON-LD match the final approval record.

STAGE GATE	A clinical and billing review confirms all claims, citations, CPT language, safety statements, dates, and attribution are accurate and appropriately qualified.

Stage 7 - Final Technical QA and Production Launch

	Run pre-launch automated validation	QA + SEO	BLOCKER

Action
Run HTML validation, structured-data validation, link checking, accessibility testing, PageSpeed/Lighthouse, security-header review, and a crawl of the staged build. Fix all critical errors and document remaining warnings.

Pass test
The release folder includes dated result links or exports. There are no broken internal links, schema errors, serious accessibility issues, mixed content, or failed form requests.

	Complete manual browser testing	QA	BLOCKER

Action
Test current Chrome, Safari, Firefox and Edge plus iPhone and Android viewport behavior. Test keyboard-only operation, zoom at 200 percent, CTA anchors, phone links, citations, portal link, accordions, and successful/failed form states.

Pass test
A signed QA matrix confirms pass/fail by browser and device. All blockers are resolved before deployment.

	Deploy without staging references	Web Dev + SEO	BLOCKER

Action
Deploy to the production URL and inspect the live response, raw source, rendered DOM, headers, metadata, structured data, social preview, assets and links. Remove noindex from production only after all prior gates pass.

Pass test
Production returns 200 with index,follow; canonical and schema use specialized-med.com; staging remains protected/noindex; source contains zero Netlify references.

	Submit and verify discovery	SEO/AEO	HIGH

Action
Add the production URL to the XML sitemap, submit the sitemap in Google Search Console and Bing Webmaster Tools, inspect the URL, request indexing, and optionally use IndexNow for supported engines.

Pass test
The sitemap returns 200 and includes the canonical URL once. Search Console sees the intended canonical and no blocking directive. Bing accepts the URL/sitemap submission.

	Monitor after launch	SEO + Analytics	HIGH

Action
Check the page at approximately 48 hours, 7 days, and 30 days for index status, selected canonical, impressions, clicks, query themes, form submissions, CTA engagement, Core Web Vitals, structured-data issues, crawl errors and staging leakage.

Pass test
A short post-launch report documents findings and owners for any corrective tasks. Search engines are selecting the production canonical and the Netlify page is not appearing as the indexed version.

STAGE GATE	The production page is live, indexable, validated, submitted, measurable, and selected as canonical while the staging copy remains unavailable to search.

Appendix A - Approved Replacement Copy

1. Title and description

```
<title>Mobile Cardiac Telemetry (MCT) | Live ECG Monitoring</title>
<meta name="description" content="Specialized Medical provides up to 30 days of MCT with live ECG transmission, attended surveillance, prescribed notifications and physician-ready reporting.">
```

2. Hero

Eyebrow: LIVE ECG STREAMING | ATTENDED SURVEILLANCE | PHYSICIAN-READY INFORMATION

H1: Mobile Cardiac Telemetry Built for Modern Cardiology Practices

Lead: Specialized Medical provides up to 30 days of ambulatory ECG monitoring with live data transmission, automatic and patient-triggered event capture, physician-prescribed notification protocols, patient support, and physician-ready reporting.

Primary CTA: Start a No-Risk MCT Pilot
Secondary CTA: Request an MCT Demonstration

3. Clinical review when no named physician review has occurred

Editorial review: Reviewed by Specialized Medical for service, workflow, and technical accuracy. This page provides general information for healthcare professionals and does not replace clinical judgment, payer requirements, emergency services, or institutional protocols.

4. CPT and reimbursement wording

Mobile Cardiac Telemetry services may involve CPT 93228 for the professional component and CPT 93229 for the technical component. Which entity may report each component depends on the services performed, contractual arrangement, payer requirements, and applicable billing rules. Coverage, prior authorization, frequency, documentation, and reimbursement vary by payer and plan. This information does not guarantee coverage, payment, or profitability.

5. Form introduction and privacy warning

Tell us about your practice, and a Specialized Medical representative will contact you regarding an MCT demonstration or pilot program.

Privacy notice: Please do not submit patient names, medical records, or other protected health information through this form.

6. Connectivity and notification wording

ECG data may be transmitted while the study is active and connectivity is available. Qualifying findings may be reviewed during the monitoring period, and notifications follow the ordering physician's selected protocol. Mobile Cardiac Telemetry does not replace emergency medical services.

Appendix B - Final Approval Checklist

[ ] Staging site is password-protected or otherwise inaccessible to public crawlers.

[ ] Staging page carries noindex while publicly accessible.

[ ] Philips hero image is removed and approved S-Patch/phone hero is installed.

[ ] Overview graphic has no unsupported outcomes or absolute claims.

[ ] No internal editorial labels or placeholder clinical-review text remains.

[ ] All CTA anchors remain on the MCT page and reach the form.

[ ] Thank-you message appears only after successful submission.

[ ] Form includes a no-PHI warning and Privacy Policy link.

[ ] Form delivery, error handling, spam controls and conversion events pass.

[ ] Production URL and one-hop redirects pass.

[ ] Canonical, Open Graph, Twitter, schema and sitemap use production URLs only.

[ ] Exactly one H1 exists and heading hierarchy passes.

[ ] Title and meta description are unique and approved.

[ ] MedicalWebPage, Service, BreadcrumbList and FAQPage schema validate.

[ ] Visible FAQ text matches FAQ schema.

[ ] OAI-SearchBot can access production content and assets.

[ ] Hero image is WebP-first, 1430 x 550, and not lazy-loaded.

[ ] Core Web Vitals and mobile layout tests show no blocker.

[ ] Accessibility scan and manual keyboard test pass.

[ ] Clinical claims and references have been verified.

[ ] CPT, CMS and payer wording is qualified and approved.

[ ] Emergency and medication-monitoring limitations remain visible.

[ ] Production is indexable; staging remains protected/noindex.

[ ] Google Search Console and Bing submissions are complete.

[ ] 48-hour, 7-day and 30-day monitoring owners are assigned.

APPROVAL AREA	NAME / DATE / EVIDENCE LINK
Web Development	
SEO/AEO	
Clinical / Billing	
Analytics / QA	
Specialized Medical Final Approval	

Appendix C - Technical Reference Standards

These references support implementation decisions. They do not replace medical, legal, coding, payer, or privacy review.

1. **Google - Block Search Indexing with noindex**
   https://developers.google.com/search/docs/crawling-indexing/block-indexing
2. **Google - Robots Meta Tag Specifications**
   https://developers.google.com/search/docs/crawling-indexing/robots-meta-tag
3. **Google - Canonical and Duplicate URL Guidance**
   https://developers.google.com/search/docs/crawling-indexing/consolidate-duplicate-urls
4. **Google - General Structured Data Guidelines**
   https://developers.google.com/search/docs/appearance/structured-data/sd-policies
5. **Google - Structured Data Introduction and JSON-LD**
   https://developers.google.com/search/docs/appearance/structured-data/intro-structured-data
6. **Google - Breadcrumb Structured Data**
   https://developers.google.com/search/docs/appearance/structured-data/breadcrumb
7. **Google - Core Web Vitals**
   https://developers.google.com/search/docs/appearance/core-web-vitals
8. **Google - FAQ Rich Result Availability**
   https://developers.google.com/search/blog/2023/08/howto-faq-changes
9. **OpenAI - Overview of OpenAI Crawlers**
   https://developers.openai.com/api/docs/bots
10. **Schema.org - MedicalWebPage**
    https://schema.org/MedicalWebPage
11. **Schema.org - Service**
    https://schema.org/Service
12. **Schema.org - FAQPage**
    https://schema.org/FAQPage
13. **CMS - Billing and Coding Article A59268**
    https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=59268

Document control

Version 1.0 | Page-specific developer handoff | Prepared October 13, 2026 | Specialized Medical LLC